

Longitudinal Care and Follow-up Plans

How to construct a closed-loop future course of action

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Document control and RAG metadata

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Retrieval rule

Retrieve when producing the future appointments and course-of-action portion of a patient review.

1. Plan construction

A complete plan links each concern to an action, owner, date/window, prerequisite, expected outcome, and fallback. The plan is individualized by an authorized clinician; the agent organizes and checks completeness.

Horizon	Required plan elements
Today	Reconcile medicines; resolve urgent warnings; document authorized directions; book immediate prerequisites
Within 72 hours	Complete high-risk access/tolerance contact or urgent testing if specifically ordered
1-2 weeks	Safety labs/observations where applicable; named clinician result review
2-6 weeks	Clinical response, symptoms, adherence, adverse effects, BP/HR/weight review as relevant
8-12 weeks	Goal-directed lab or imaging review where authorized; medication reconciliation
3-6 months	Cardiology review, surveillance, prevention goals, ongoing monitoring cadence
Annual	Full structured review, diagnoses/medicines/allergies reconciliation, care-plan renewal

2. Appointment types

Appointment	Purpose	Prerequisites
Pharmacist medication review	Reconciliation, interactions, instructions, adherence barriers	Current medication list, allergies, recent labs, signed plan
Cardiology clinician review	Diagnosis/response assessment and authorization	Relevant observations, tests, medication review
Nurse monitoring visit	Observations, education, symptom review	Clear protocol and escalation thresholds
Diagnostics appointment	Obtain ordered evidence	Complete order and test-specific preparation
Results review	Interpret and decide action	Final result, prior comparison, current symptoms/medicines
Annual structured review	Update full longitudinal plan	Problem list, medication/allergy reconciliation, surveillance status

3. Goals and outcomes

- Use measurable goals selected by the clinician, not model-created targets.
- Separate symptom goals, observation/laboratory goals, prevention goals, and patient priorities.
- Record barriers such as access, health literacy, affordability, dexterity, swallowing, cognition, and transport.
- Document shared decision-making, declined options, and agreed alternatives without judgment.

- Close the loop: planned action -> completion -> result -> review -> communication -> next plan.

4. Final plan template

Issue: [patient fact + timestamp]. Authorized action: [signed plan or 'awaiting authorization']. Exact instructions: [complete order-derived wording]. Warning/hold/escalation: [specific policy]. Test: [what/why/date/owner]. Appointment: [type/window/prerequisites/owner]. Outcome communication: [channel/time]. Evidence: [citations].

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